AC-10 Post-Resuscitation Care

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use after sustained return of spontaneous circulation in an adult after cardiac arrest.

Post-ROSC stabilization and destination

1. Confirm sustained ROSC	Pulse, BP/MAP, organized rhythm, and perfusion; keep pads attached and prepare for re-arrest.
2. Oxygenation	100% oxygen until a reliable SpO2, then titrate to 90-98%. Use waveform capnography with an advanced airway.
3. Ventilation	Begin near 10 breaths/min and target EtCO2 approximately 35-45 mmHg; avoid hyperventilation.
4. MAP at least 65?	If low, NS 250-500 mL IV/IO and reassess; use smaller volume for CHF, renal/liver failure, or volume overload.
5. Persistent hypotension	Norepinephrine 0.1-2 mcg/kg/min IV/IO, titrated to MAP at least 65 mmHg; alternatives require Medical Control.
6. ECG + neuro + temperature	Obtain/transmit 12-lead as feasible, treat glucose below 70, document GCS/pupils/seizures, and prevent fever. No rapid cold-fluid cooling.

Key treatment and handoff

Item	Approved action
Destination	PCI-capable center for STEMI, cardiogenic shock, recurrent ventricular arrhythmia, or ongoing ischemia; consider Covenant Health Air when appropriate.
Re-arrest	Immediate return to AC-03/AC-09.

AC-13 supplements this protocol. Stabilize airway, ventilation, oxygenation, perfusion, ECG, glucose, and cause-directed care before temperature management.

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SAFETY, LINKED PATHWAYS, AND DOCUMENTATION

Safety checks

- Avoid hyperoxia after reliable SpO2 monitoring and avoid hyperventilation.
- Do not routinely initiate rapid cold IV-fluid cooling; do not rapidly actively rewarm a spontaneously hypothermic patient.
- Repeat ECG if the immediate post-ROSC tracing is nondiagnostic or the condition changes.

Linked pathways

Pathway	Use when
AC-04	STEMI/ischemia
AC-13	Post-arrest temperature management
AR-03	Advanced-airway management

Documentation and release control

- Document the presenting condition, focused findings, rhythm/ECG when applicable, all interventions with dose/route/time/response, serial vital signs, reassessments, destination decision, and notifications.
- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.
- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.